This Patient Group Direction (PGD)is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Metronidazole gel or Azelaic acid for the treatment of Rosacea

Patient Group Direction, version 006, issued 30 September 2026. Supersedes Version 005, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE / NICE CKS Guidelines for Rosacea

Overview

- **Rosacea** is a chronic, relapsing skin condition that affects the **central face**, characterised by **flushing**, **persistent erythema**, **papules/pustules**, and sometimes **telangiectasia**, **eye symptoms**, or **skin thickening** (phymatous rosacea).
- Most common in **fair-skinned adults aged 30–60**, especially women, though **rhinophyma** is more common in men.

Subtypes of Rosacea

1. **Erythematotelangiectatic** – persistent redness, flushing, visible vessels.

2. **Papulopustular** – acne-like spots on red background.
3. **Phymatous** – thickened skin (e.g. rhinophyma).
4. **Ocular** – dry, sore, gritty eyes, blepharitis.

Many patients have overlapping features.

Assessment

- Diagnosis is clinical based on symptom pattern and distribution.
- Rule out other conditions (e.g. acne, seborrhoeic dermatitis, lupus).
- Ask about **triggers**: sun exposure, alcohol, spicy food, stress, temperature extremes, skin products.

Management

1. General Measures

- Identify and **avoid triggers**.
- Use **non-irritant, non-comedogenic skincare**.
- Daily use of **broad-spectrum sunscreen (SPF 30+)**.
- Avoid steroid creams unless prescribed short-term by a specialist.

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2. Pharmacological Treatments (Tailored to Symptoms)

Symptom	First-Line Treatment	Notes
Erythema/flushing	**Brimonidine gel** (0.33%)	Vasoconstrictor; apply once daily
Papules/pustules		

- **Topical metronidazole 0.75%/1%**
- **Topical azelaic acid 15–20%**
- **Topical ivermectin 1%** | Apply once or twice daily for up to **8–12 weeks** |
  | **Moderate-severe or unresponsive papulopustular rosacea** | **Oral tetracyclines** (e.g. doxycycline 100 mg OD) | Use for **6–12 weeks** |
  | **Ocular rosacea** |
- **Artificial tears**
- **Oral doxycycline** (if moderate/severe) | Refer to ophthalmology if severe or vision is affected |

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3. Specialist Referral

Refer to dermatology if:

- Diagnosis is uncertain
- Severe or treatment-resistant disease
- Phymatous rosacea (may require surgery/laser)
- Significant **psychosocial impact**

Follow-Up

- Review after **8–12 weeks** of treatment.
- Taper or stop treatment if symptoms are controlled.
- Long-term maintenance with topical agents may be needed.

Patient Group Direction

for the supply/administration of

Metronidazole gel

for the treatment of

Rosacea

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Metronidazole gel is indicated for the topical treatment of rosacea in adults, particularly for reducing inflammatory lesions such as papules and pustules.
Inclusion criteria	- Adults aged 18 years and over. - Diagnosis of mild to moderate rosacea with inflammatory lesions. - Informed consent obtained. - No contraindications to metronidazole.
Exclusion criteria	- Known hypersensitivity to metronidazole or other nitroimidazoles. - Severe rosacea requiring systemic treatment. - Broken, irritated, or eczematous skin. - Pregnancy or breastfeeding. Refer to the GP.
Cautions	- Avoid contact with eyes, mucous membranes, and broken skin. - Use sunscreen and avoid excessive sunlight exposure. - Wash hands after application. - Monitor for prolonged use; review at 8 weeks for effectiveness.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Metronidazole gel 0.75% w/w
Legal category	POM
Storage	Store below 25°C. Do not freeze.
Route/method of administration	Topical application to affected areas of the face.
Dose and frequency	Apply a thin layer to the affected area twice daily.
Quantity to be administered and/or supplied	One 30 g tube per supply; up to 2 x 30 g per 8-week course. Record each supply.
Maximum or minimum treatment period	Course of up to 8 weeks under this PGD (2 x 30 g); review at 8 weeks. Any further course only after that review, and refer to the GP if there is no improvement at 8 weeks or if use would exceed 12 weeks in total.
Adverse effects	Dryness, skin irritation, stinging, redness, metallic taste. Rare: peripheral neuropathy with prolonged use. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply, dose, form, route and quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken ● that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	- To seek medical advice if symptoms worsen rapidly or significantly, if there is no improvement at the 8-week review, or they become systemically very unwell. Improvement may take several weeks. - Advise on the importance of avoiding trigger factors wherever possible. - Suggest that a diary may be helpful to identify stimuli and triggers that may exacerbate rosacea. - Advise on the importance of effective sun protection and to avoid the use of sunbeds. - Advise on the use of regular non-oily emollients if the skin is dry and the possible use of yellow- or green-tinted cosmetics to help camouflage skin erythema.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and

administration of medicines BNF (British National Formulary) | NICE ● Summary of Product Characteristics for drug

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
30/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD	13th December 2025

Patient Group Direction

for the supply/administration of

Azelaic Acid

for the treatment of

Rosacea

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Azelaic acid is indicated for the topical treatment of mild to moderate papulopustular rosacea in adults.
Inclusion criteria	- Adults aged 18 years and over. - Diagnosis of papulopustular rosacea suitable for topical therapy. - Informed consent obtained. - No contraindications to azelaic acid.
Exclusion criteria	- Known hypersensitivity to azelaic acid or any of the excipients. - Severe rosacea requiring systemic treatment. - Broken, irritated, or eczematous facial skin. - Pregnancy or breastfeeding. Refer to the GP.
Cautions	- Avoid contact with eyes, mouth, mucous membranes, and broken skin. - Wash hands after use. - Use sunscreen and avoid excessive UV exposure. - Monitor for skin irritation; discontinue if severe or persistent. - Worsening of asthma in patients treated with azelaic acid has been reported.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Azelaic acid 15% gel
Legal category	POM
Storage	Store below 25°C. Do not freeze.
Route/method of administration	Topical application to affected areas of the face. Occlusive dressing or wrappings should not be used, and hands should be washed after applying the gel
Dose and frequency	Apply a thin layer to the affected area twice daily (morning and evening). 2.5cm (1 inch) of gel is sufficient for the entire facial area. In the event of skin irritation, the amount of gel per application should be reduced or the frequency of use should be reduced to once a day until the irritation ceases. If required, the treatment should be temporarily interrupted for a few days.
Quantity to be administered and/or supplied	One 30 g tube per supply (about 4 to 5 weeks at 2.5 cm twice daily, per the SmPC); up to 3 x 30 g per 12-week course. Record each supply and review at 8 to 12 weeks.
Maximum or minimum treatment period	Initial course of up to 12 weeks; reassess ongoing need periodically. The duration of use can vary from person to person and also depends on the severity of the skin disorder. In general, a distinct improvement

	becomes apparent after 4 weeks of treatment. To obtain optimum results, Azelaic Gel can be used over several months in accordance with the clinical outcome. If there is no improvement after 2 months or a new exacerbation of rosacea, Azelaic Gel should be discontinued, and other therapeutic options should be considered.
Adverse effects	Burning, stinging, itching, dryness or redness at application site. Rare: hypopigmentation or hypersensitivity reactions. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply, dose, form, route and quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken ● that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	- To seek medical advice if symptoms worsen rapidly or significantly, if there is no improvement after 2 months of regular use (a distinct improvement is generally apparent after 4 weeks), or they become systemically very unwell. - Advise on the importance of avoiding trigger factors wherever possible. - Suggest that a diary may be helpful to identify stimuli and triggers that may exacerbate rosacea. - Advise on the importance of effective sun protection and to avoid the use of sunbeds. - Advise on the use of regular non-oily emollients if the skin is dry and the possible use of yellow- or green-tinted cosmetics to help camouflage skin

	erythema.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines BNF (British National Formulary) | NICE
- Summary of Product Characteristics for drug

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
30/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD	13th December 2025

Version and change record

Version: v006

Issued: 30 September 2026

Supersedes: Version 005, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v005

Issued: 14 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Exclusion criteria, both arms: pregnancy or breastfeeding is an exclusion with referral; the prescriber wording is removed because there is no prescriber in a PGD supply.

Metronidazole cautions and treatment-period row: the course under this PGD is 8 weeks with review at 8 weeks, matching the 2 x 30 g quantity; the 8 to 12 week ambiguity is removed.

Follow-up advice: the 3 to 4 week infection-template line is replaced by each arm's own review point (metronidazole 8 weeks; azelaic acid 2 months).

Records row: the two run-together bullets were separated and punctuated (date of supply, dose, form, route and quantity supplied; adverse reactions and actions taken; supplied via PGD).

Records row: the under-18 record-retention bullet was removed because this PGD does not treat anyone under 18.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 13 December 2025

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

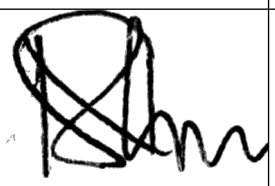
Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027.

Azelaic acid quantity: one 30 g tube covers about 4 to 5 weeks at the SmPC application (2.5 cm, about 0.5 g, twice daily) of a course the PGD runs for 12 weeks. Quantity now up to 3 x 30 g per 12-week course, one tube per supply, with the review at 8 to 12 weeks. Metronidazole gel quantity stated the same way for its 8-week course.

Signed on behalf of Get Real Health

Version v006, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.